



Adrabetadex U.S. Launch Background and Core Assumptions

MLR review draft

Audience: Medical, legal, regulatory, and internal access reviewers

Prepared March 30, 2026. Facts reflect the attached source set and public CMS, HRSA, Medicaid, Medicare, and payer policy sources reviewed on March 30, 2026.

Launch frame

Adrabetadex is under FDA priority review with a Prescription Drug User Fee Act action date of August 17, 2026. The launch planning frame in the supplied Beren materials centers on infantile-onset Niemann-Pick disease type C (NPC), defined internally as neurologic symptom onset before 6 years of age.

The epidemiology used across the source set is consistent. The U.S. PIE deck places total NPC incidence at roughly 40 patients per year in the United States, infantile-onset incidence at roughly 20 patients per year, and infantile-onset incidence at roughly 0.059 patients per year per 1 million covered lives. The same deck states that about 50% of the NPC population has infantile-onset disease.

The burden narrative is also stable across the attachments. The PIE deck reports an average delay between symptom onset and confirmed diagnosis of 2.5 years for early infantile-onset NPC and 4.3 years for late infantile-onset NPC. In a disease where early infantile mortality can occur before first grade, that delay is economically relevant because it compresses the usable treatment window and concentrates payer review on very sick children.

Clinical differentiation in the supplied evidence package

The current adrabetadex value story is built around survival and preservation of function, not around symptom control alone. The PIE materials state that treatment with adrabetadex was associated with a 71% reduction in risk of death in infantile-onset NPC versus matched external controls, with 5-year survival of 84% in treated patients versus 42% in external controls.

The same source set reports that the signal persists across major subgroups. The early infantile subgroup is described as having an 85% lower risk of death versus controls and the late infantile subgroup a 66% lower risk of death. The deck also reports benefit with and without concomitant miglustat, which matters because current NPC practice still uses miglustat off label in the United States.

The safety package in the internal training deck reports 71 patients treated on the proposed commercial dosing regimen for an average of 3.8 years, with 2 of 71 patients discontinuing because of a treatment-emergent adverse event. That same source identifies hearing impairment, transient post-dose fatigue or ataxia, and vomiting as the recurrent management issues.

Commercialization assumptions already embedded in the launch model

The Beren internal training deck makes the commercial assumptions explicit. It states that U.S. payer primary research supports premium pricing up to \$1.8 million, estimates annual list prices for Aqneursa at \$350,000 to \$700,000 and for Miplyffa plus miglustat at \$590,000 to \$1.7 million, and models adrabetadex with a weighted average gross-to-net discount of about 15% in a 50% commercial and 50% Medicaid mix.

The same base case assumes a \$1.1 million annual price, a 1.0% annual price increase, 175 currently diagnosed infantile patients, 60 expanded access patients converting at launch, 250 undiagnosed patients, 20 misdiagnosed patients, and 25 newly born infantile patients per year. Penetration assumptions are 80% of diagnosed patients, 65% of undiagnosed patients, 45% of misdiagnosed patients, and 75% of newly born patients.

The provider model brief anchors unit economics with a working WAC of \$39,000 per 900 mg vial administered every two weeks. At 26 administrations per year, that equals \$1,014,000 of annual gross drug WAC before administration costs, wastage, or pricing round-up. The aligned client set tests 15%, 20%, and 30% commercial reference-price views, or \$33,150, \$31,200, and \$27,300 per dose, and 5% and 10% non-340B manufacturer support, or \$1,950 and \$3,900 per dose, because those ranges change provider margin outside 340B.

How commercial payers already handle analogous rare neurologic therapies

- UnitedHealthcare's commercial Miplyffa criteria require genetically confirmed NPC, use in combination with miglustat, no concurrent Aqneursa, specialist involvement, and documentation of positive clinical response at reauthorization. Authorizations are issued for 12 months.
- UnitedHealthcare's commercial Aqneursa criteria require genetically confirmed NPC, treatment of neurologic manifestations, use with miglustat or prior failure, contraindication, or intolerance to miglustat, no concurrent Miplyffa, specialist involvement, and positive clinical response at reauthorization. Authorizations are issued for 12 months.
- Louisiana Blue's Miplyffa policy adds harder functional gates. It requires age 2 years or older, weight at least 8 kg, neurologic signs, use with miglustat, no concurrent Aqneursa, no adult-onset NPC, and retained ambulation independent or assisted. Reauthorization requires improvement or stabilization in ambulation, fine motor function, swallowing, or speech.
- Aetna's Spinraza policy shows how a major payer manages intrathecal rare disease therapy under the medical benefit. It requires genetic documentation, baseline motor assessments, evidence of positive response at continuation, and recognizes image-guided intrathecal administration as medically necessary.



- Aetna's Brineura policy requires administration by or under the direction of a physician knowledgeable in intraventricular administration and continuation based on no loss or slowed loss of ambulation from baseline.

What these assumptions mean for MLR review

The launch case does not rest on a blank-check medical benefit environment. It rests on a market in which commercial payers already cover rare neurologic therapies through prior authorization, specialist requirements, continuation rules, and combination-therapy restrictions. The payer question is not whether infantile NPC is serious. The payer question is how tightly the plan will operationalize access.

The source materials therefore support a clean internal distinction between clinical differentiation and reimbursement execution. The clinical story centers on survival. The reimbursement story centers on documentation, coverage criteria, coding, channel control, the concentration of volume inside a small number of experienced pediatric specialty centers, and the HCPCS and pass-through workstream that can first become operational on January 1, 2027 if the approval and pricing package is complete for CMS's September 2026 MEARIS cycle.

For MLR purposes, the defensible launch position is that adrabetadex enters a supportive but tightly managed access environment. The source set does not support frictionless access language, and it does support explicit commercial and Medicaid operational planning. The aligned client materials also show that non-340B buy-and-bill economics remain thin, 340B economics are materially better, and white-bagging removes drug spread entirely.

Sources

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